

iGel Airway

Supraglottic airway placement is a valuable skill in the prehospital setting. Proper utilization of this device has demonstrated superior airway management to endotracheal intubation when successfully placed and monitored due to easy of use, success of placement, adequate ventilation, and decreased risk of unintentional extubation.

Indications for supraglottic airway management with iGel

- Apnea
- Cardiac arrest
- Failed intubation

Contraindications

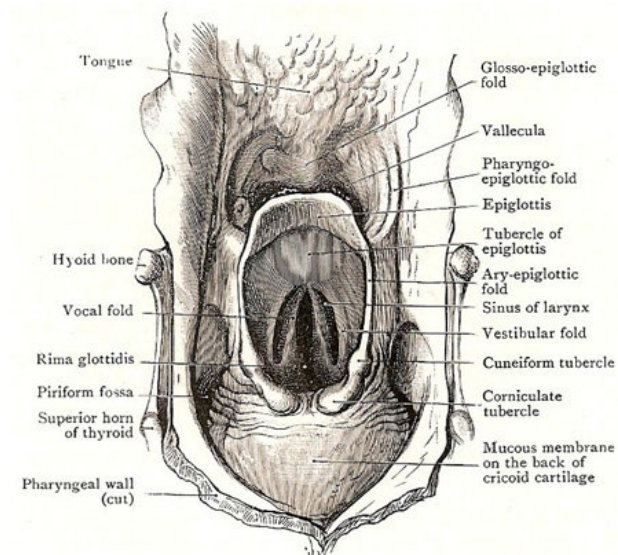
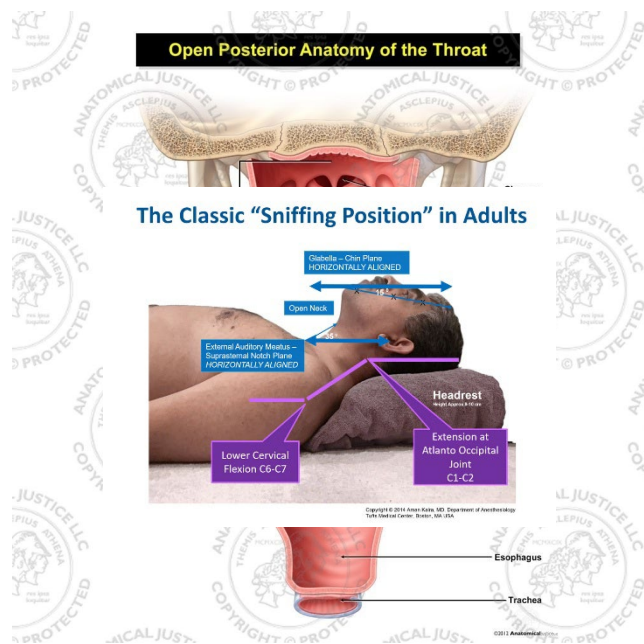
- Gag reflex
- Caustic ingestion or extensive airway burns
- Inadequate ventilation/oxygenation with the iGel device

Equipment needed for supraglottic airway management

- Appropriate size iGel – sized by ideal body weight (height)

Height in Inches (feet)	Ideal Body Weight (Kg)		
	Male	Female	
60 (5'0")	50	45	iGel Size 3 (Adult Small)
61 (5'1")	52	48	iGel Size 4 (Adult Medium)
62 (5'2")	55	50	
63 (5'3")	57	52	
64 (5'4")	59	55	iGel Size 5 (Adult Large)
65 (5'5")	61	57	
66 (5'6")	64	59	
67 (5'7")	66	62	
68 (5'8")	68	64	
69 (5'9")	69	66	
70 (5'10")	73	68	
71 (5'11")	75	71	
72 (6'0")	78	73	
73 (6'1")	80	75	
74 (6'2")	82	78	
75 (6'3")	84	80	
76 (6'4")	87	82	
77 (6'5")	89	85	
78 (6'6")	91	87	
79 (6'7")	94	89	
80 (6'8")	96	91	

Anatomy for delayed sequence intubation



Position for Intubation – ear to sternal notch

Supraglottic Airway Management - iGel

1. Choose the appropriate size iGel based on ideal body weight – see chart
2. Apply a water-based lubricant to the posterior aspect of the iGel
3. Position the head in the sniffing position (unless c-spine injury is suspected)
4. Hold the iGel along the bite block with the dominant hand. With the non-dominant hand, hold the mouth open and stabilize the tongue against the mandible
5. Introduce the leading soft tip into the mouth of the patient toward the hard palate
6. Glide the device downwards and backwards along the hard palate with a continuous, but gentle push until the bite block reaches the teeth
7. Allow the iGel to reseat itself as you release your grip. The tip of the iGel should be located in the upper esophageal opening and the cuff should be located against the laryngeal framework
8. The incisors should be resting on the bite block
9. Confirm proper placement and positioning by auscultation of bilateral breath sounds and waveform capnography
10. Secure the iGel using the included strap
11. Verify placement with ETCO₂ after every patient transfer.

NOTE

- **If the ETCO₂ is 0, the iGel is occluding the airway – REPOSITION!!!**
- **Inability to ventilate the patient after placement suggests iGel is positioned improperly – REPOSITION!!**
- If unable to place the iGel after 2 attempts, discontinue the procedure and continues ventilations with BVM/NPA/OPA – **consider delayed sequence intubation and/or failed airway guideline**
- The iGel does not protect the airway from the effects of regurgitation and aspiration – elevate head of bed to 15-30 degrees if the patient tolerates this elevation
- A suction port in the iGel allows a soft tip suction catheter to be placed in the stomach through the tube